

Q&A: Halloween candy, sodium, breastfeeding and colon cancer, and placenta previa

Ask ParentData



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27



14



1

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It's Q&A day!

Today's first question is available to all subscribers (about runners' sodium needs), and there are a few bonus ones behind the paywall (about Halloween candy eating, breastfeeding and colon cancer, and placenta previa and sexual activity).

Enjoy! And remember, you can [submit questions for future weeks here](#).

I'm a runner and have heard recommendations for an electrolyte drink called LMNT. One serving has 1,000 milligrams of sodium. On their website, they talk about how the FDA guidelines are too low and boast about their studies showing we need more sodium based on medical data from various cultures. Curious about their claims. Is there any reliable science on this?

I didn't write this question, but I could have. So I asked the person who I rely on for this information to weigh in. Thanks to Meghann Featherstun for the answer! —Emily

This is a great question and something that confuses a lot of us. [I am a sports dietitian](#)

guide our nutrition choices. They present evidence that consuming 2,300 milligrams of sodium a day for adults represents the safest amount to reduce chronic disease risk. Yet they also state that an adequate intake (AI), aka the amount of sodium we need to survive, for most adults is only 1,500 mg a day.

However, as [runners](#), we have higher sodium needs than sedentary adults, and consuming the above amounts of sodium may not be optimal for us. When we sweat, we lose water and electrolytes such as calcium, magnesium, potassium, and sodium. Sodium is lost in the largest amounts; the average runner loses 900 mg of sodium per liter of sweat. It is not uncommon for runners to lose one to two liters of sweat per hour in the heat or on a treadmill. If you go on a two-hour-long run, that means somewhere between 1,800 and 3,600 mg of sodium lost. That's a lot — and it needs to be replaced.

It is important to replace the sodium lost via sweat in our daily diet to stay adequately hydrated and support overall health and running performance. Therefore, as a runner, you may need an additional 1,000 to 2,000 or more mg of sodium above the recommended DRI. If we do not increase our sodium intake as runners, side effects can include headaches, dizziness, fatigue, nausea, diarrhea, and muscle cramps.

The takeaway here is that our sodium needs are highly individualized based on our activity level, sweat rate, sweat composition, and overall health status and concerns.

What's the data on Halloween candy? My daughter is about to turn 3, and this is the first year where I could imagine her joyfully eating a mountain of tiny candy bars. Do I need to worry about the sugar? The food dyes? The tears when I tell her she doesn't get to eat a pile of candy the next day?

—Halloween How-to

There are two angles on this question, in my view. The first is how cautious, in general, we should be about sugar and candy. The second is whether Halloween is an exception.

On the first question, this is a deep issue with many feelings on both sides. I've written often in the newsletter about the evidence we have on nutrition, about [BMI and what it means](#), about [weight loss drugs](#). I've [talked to Virginia Sole-Smith](#) about her work on these topics. What I can say based on all those posts is that this is a very, very fraught

set of conversations.

There are two pieces of data I think are relevant. Fact one: when foods are forbidden, they become more interesting to kids. Never allowing your child cake makes cake fascinating. This is a downside of restricting particular foods. On the other hand, we have a lot of evidence that taste is formed in childhood. Exposure to a wide variety of foods and flavors makes those more palatable long-term. Kids (and other people) are usually more drawn to sugar than to vegetables. As a result, developing a taste for a variety of foods is likely dependent on some curation of how kids eat.



Getty Images

Putting these together, my read is that (outside of Halloween) the evidence would not support completely restricting candy, nor would it support allowing completely free rein over candy at any time. It's a balance, which every family will find differently.

As to Halloween, though ... there is absolutely no evidence that allowing a single day of unlimited candy consumption will negatively impact your children beyond a

stomachache. The idea of a “sugar high” for kids is a myth (yes, your kid is a jerk after a birthday party, but that’s not the sugar, it’s the overstimulation). Red dye doesn’t make kids hyperactive ([says the data](#)). Halloween is fun. Do you have to offer unlimited candy? No. But if you do want to embrace the joy of a day of kids making themselves a little sick with mini Snickers bars, please feel supported in doing so.

I just read that [a study was published](#) that breastfeeding increases the risk of colorectal cancer?! Should I stop??

—Concerned exclusively-BF mom

We are all very used to panic headlines about the [dangers of not breastfeeding](#). I spend a good deal of my time debunking these claims. The latest breastfeeding panic headline is the opposite — the claim in this paper is that breastfeeding *increases* the chance that a child later develops colon cancer. The authors are quick to note that this doesn’t mean you shouldn’t breastfeed, since there are other benefits. But the scare is there: did your choice to breastfeed actually make your child’s long-term health worse?

The paper in question is [here](#). The analysis is relatively straightforward. The authors use data from a large, well-known study called the Nurses’ Health Study, which enrolled a large cohort of female nurses beginning in the 1990s. These women have been followed through at least 2015, with information collected about their health status over that period; this is how the researchers learn about colon cancer. Women in the study are also asked if they were breastfed and, if so, for how long.

The primary finding in the study is that the women who reported being breastfed were more likely to get colon cancer than others. The increase is about 20% to 40%, which sounds large but is small in absolute terms — in the data, they see 1,490 cases in 3.5 million person-years of follow-up.

Given the information the authors had available, this study is well-done (in my view). The sample is large, and the analysis makes sense. To the extent possible, the authors adjust for differences across the groups in terms of demographics.

Having said that, there are a few issues. One they highlight is that the breastfeeding data is self-reported by the individual, which introduces error. This is an example of

something that seems like it could be a problem but I think isn't very important — yes, there is statistical noise, but it's not likely to impact the results.

A second issue is that there are important demographic differences between the moms who breastfed and those who did not; as in virtually all studies of this type, the breastfeeding mothers are better-educated and have more resources than the non-breastfeeding ones. The way that might impact the results is a little subtle. Generally, more resources are associated with *better* health, which would push against their findings. In this case, though, I worry a bit about variation in screening. If people with more resources are more likely to be screened, and then more likely to be diagnosed, we might see these results even if the actual incidence of cancer is the same. (This happens with breast cancer screening.)

There are some additional analyses that might be helpful here: a more complete look at screening behaviors, and maybe some placebo tests using other behaviors. At this point, I think this is the range of something to look into more, not something to take to the bank (a point I think the authors would agree with).

A final note: I cannot help but be struck in reading this paper and the commentary on it by how differently things are reported here, versus in similar papers where the finding is in favor of a benefit of breastfeeding. The authors here are tripping over themselves to say that this doesn't indicate that breastfeeding has risks and to emphasize that this shouldn't change breastfeeding behavior. In a sense, I do not disagree, given where the evidence is, but ... it's notable given the rhetoric I'd expect if the findings were the opposite.

I've been diagnosed with a complete placenta previa (at 20 weeks), and it is continuing while at 26 weeks now. I was put on full pelvic rest by my perinatologist, and while I understand the seriousness of the potential outcomes of this condition, my husband and I can't seem to find any tangible information about why this is the case. I'm asking for selfish reasons — I miss the intimacy with my husband, and I've been feeling great during my second trimester, so I'm reluctant to let it go by as I move closer to the third trimester, when I will inevitably be bigger, more tired, and feeling less like having sex or engaging in intimate activities. But what are the real risks involved with sexual activity, even non-intercourse, for this condition?

I should also add that we conceived via IVF, so in general we are being very cautious during this pregnancy given our difficulties to get here, but this is one area where I personally feel a real impact to my (and my husband's) quality of life, and there doesn't seem to be much evidence to support it.

—Anonymous

[Placenta previa](#) is a condition in which the placenta is situated over the cervical opening, either partly or fully. The major concern with the condition is bleeding — if it persists until delivery, you will need a cesarean section, since a vaginal delivery would be extremely dangerous to you and the baby. The main concern *during* pregnancy is also severe bleeding.

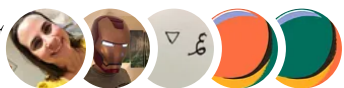
Before we get into sex: let's briefly touch on the resolution of this condition. Because of uterine and fetal growth, placenta previa that is diagnosed early in pregnancy is very likely to resolve. [Among those who are diagnosed at 20 weeks](#), about 90% resolve by the time of delivery. If the previa persists to 26 weeks, as yours has, there is still about a 50% chance that it will resolve before delivery. When someone still has previa at 32 weeks, doctors then begin to assume that it will be present at delivery (and they will plan a C-section).

The concerns about bleeding mean that there are a number of restrictions put on behavior. You will not get digital cervical exams, where a doctor sticks their gloved fingers through the cervix to measure dilation. In addition, there is a recommendation of no sex. The concern here is twofold. First, sex can lead to uterine contractions, which might prompt bleeding. Second, the penetration of the penis could effectively bump up against the cervix, leading to bleeding. Unfortunately, the restrictions here extend to any sex leading to orgasm for you, due to the uterine contractions.

There is no actual data suggesting whether this happens and how often. The recommendation is based on theory and extrapolation from the dangers of digital exams through the cervix.

This is a complicated one — I hear you about second-trimester sex — but there is some biological basis for this restriction. But no actual evidence.

What to do? I can't tell you! What I will leave you with is that you can have intimacy without sex, and maybe this is an opportunity to explore that. Also, your husband can have orgasms. I don't know if that's especially comforting, though. Where's the equality? I'm hoping for you that the previa resolves by the next ultrasound and that the third——



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